

Ear - Foreign Body - Female (Adult)

Algorithm 1158 | Adult (18 years and older) | Gender at birth: Female

Definition

- Safety-first assessment of a suspected foreign body in the external ear canal, prioritizing button batteries, sharp objects, penetrating trauma, bleeding, and complications from attempted removal.
- Adult patient.
- Female.

Initial assessment

1. [OPEN_TEXT] What may be in the ear: a button or coin battery, magnet, sharp object, food or seed, insect, bead, cotton, hearing-aid part, or unknown object? Do not delay care trying to identify it.
2. [DURATION] When was it inserted or first noticed, and has anyone attempted removal, irrigation, oil, drops, glue, tweezers, cotton buds, or another tool?
3. [YES_NO] Is there severe pain, active bleeding, discharge, sudden hearing loss, dizziness, vomiting, facial weakness, or suspected eardrum injury?
4. [OPEN_TEXT] Record age, affected ear, whether the object is visibly protruding, and whether this could involve coercion, abuse, self-harm, or an unsafe caregiver.

Triage and redirect questions

1. Redirect → Earache

Is there no object in the ear, with ear pain or pressure now the main concern?

Ear pain without a foreign body belongs in the Earache pathway.

1. Disposition level 100

Is the object a button or coin battery, paired magnet, sharp or penetrating object, or unknown object with severe pain, active bleeding, sudden major hearing loss, severe dizziness, facial weakness, or reduced responsiveness?

Button batteries can rapidly cause corrosive tissue injury; magnets, sharp objects, penetrating trauma, bleeding, and neurologic or vestibular symptoms require immediate specialist-capable emergency assessment.

1. Disposition level 70

If no emergency feature is present, is any object or insect still in the ear, or are pain, discharge, hearing change, failed removal attempts, or uncertainty present?

A retained ear foreign body requires prompt direct examination and controlled removal to avoid canal or eardrum injury. The exact Qatar service and timeframe are governance-required.

Advice

Emergency hazardous ear foreign body

- Call Qatar 999 now for a hazardous object or serious symptoms and follow the call-handler's transport instructions. Do not put liquid, oil, drops, food, or tools into the ear; do not irrigate; and do not pull an object unless it has already fallen completely free. Keep the patient still and do not allow self-driving. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening pain or bleeding; dizziness, vomiting, facial weakness, reduced responsiveness, or sudden hearing loss.

Foreign body in ear - controlled in-person removal

- Use the Qatar governance-approved in-person service and timeframe. Do not attempt blind extraction with fingers, cotton buds, tweezers, glue, suction, or other tools, and do not irrigate or add oil or drops because the object, eardrum status, and material may be uncertain. Stop further attempts after any failed removal and keep the ear dry.
- Call Back If: pain worsens; bleeding starts; hearing suddenly changes.

First aid

- Call Qatar 999 now for a hazardous object or serious symptoms and follow the call-handler's transport instructions. Do not put liquid, oil, drops, food, or tools into the ear; do not irrigate; and do not pull an object unless it has already fallen completely free. Keep the patient still and do not allow self-driving. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Earache", <https://www.nhs.uk/conditions/earache/> (page last reviewed 27 October 2025)
- US National Library of Medicine, StatPearls, "Ear Foreign Body Removal", <https://www.ncbi.nlm.nih.gov/books/NBK459136/> (accessed 25 July 2026)
- Qatar Ministry of Public Health, "Healthcare Services in Qatar", <https://sportandhealth.moph.gov.qa/EN/faninfo/Pages/HealthcareServicesInQatar.aspx> (999 ambulance access; accessed 25 July 2026)

Provenance

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